

Student Workbook



V2

WELCOME TO THE
NCLEX VIRTUAL TRAINER



The Future of Nurse Education

Author, Educator, Mentor | Regina M. Callion CEO, MSN, RN

The ReMar Review NCLEX® Virtual Trainer Workbook

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A MESSAGE FROM YOUR INSTRUCTOR



Your success in nursing will be determined by your ability to think, plan, decide, and take action. The actions you take will be based upon your core content knowledge of the fundamental practice of nursing.

These same skills are necessary as you prepare to take NCLEX®. The stronger you are with the fundamentals, the faster you will learn how to critically think and make the right decisions.

Welcome to the ReMar Nurse NCLEX Virtual Trainer the future of nursing education. My name is Regina M. Callion MSN, RN and I will be your instructor on this journey. I started my nursing career very early, at the age of 16 I was presented with the challenge to take care of my grandparents in their home.

My grandmother was a double amputee she lost her legs and vision to diabetes. My grandfather had a stroke and couldn't talk, swallow or walk. Some people might think that a teenage girl would feel helpless but I was empowered to provide care. It was the home health nurse Linda who taught me simple, straight to the point nursing information. She help me turn my challenge into an opportunity. She wasn't afraid of my age or lack of experience. She believed in me. As you begin our journey together I want to let you know that you are on the right track and I want you to see this challenge of passing NCLEX as the opportunity of a lifetime.

I need you to put pride to the side, cast away doubt, feelings of inadequacy, and any other thought that does not support this one effort here and now. I have personally beaten the odds and as a ReMar Nurse I expect you to do the same. As a company ReMar Review has helped thousands of registered nurses pass NCLEX and they are now living out their dreams. Whether this is your first time taking NCLEX or you've tested 10X or more, I want to encourage you to stay focused on this one goal – of believing in yourself! I know that you can pass NCLEX because if seen it done so many times and the only thing they needed to do was to follow the instructions, study the content, and to not give up. You are joining a global community of ReMar nurses who are committed to doing the work of passing NCLEX and getting their nursing license.

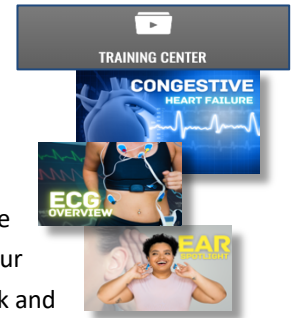
I know passing NCLEX can be difficult and I'm so glad that you decided not to do it alone. My goal is to help you study the content and make this process as simple as possible. I want to take what's in head and put it into yours. Stay focused; put FAITH over fear and continue to invest in yourself because YOU CAN, YOU WILL, and YOU MUST Pass NCLEX! Scan the code below and let's stay connected!



HOW TO USE THE NCLEX-VT STUDENT WORKBOOK

This workbook is designed to be used in combination with the ReMar Nurse NCLEX Virtual Trainer online platform. You can also choose to sign in using your Google or Facebook account or create your own username and password by visiting your NCLEX-VT System at **VT.ReMarNurse.com**

Your homepage will be the training center and you will have many icons on the left side menu. Let's start with the Training Center. In your training center you will find all of your lecture content. This is where you will also be held accountable for completing homework and practice exams.



When you click on the training center icon you will see your NCLEX information presented in multiple rows of lesson icons. Click on the select button to open up the lessons within a row. You will cover each lesson in sequential order. This is where you will watch your NCLEX content lectures as you fill-in the blanks to this workbook.

Be sure to use your daily study calendar to keep your training progress on track!

If you want to be an active member of the community take the time to start a discussion about the lesson you are watching. Your fellow ReMar Nurses may have valuable insight to share back at you as you now study together. Remember this program gives you unlimited online access with auto-renewal should you need more time.

HOW TO USE YOUR FOUR-WEEK NCLEX STUDY CALENDAR

When you log into your NCLEX Virtual Trainer the first thing need you to do is visit the File Vault. This is where you will find your detailed four-week NCLEX Study Calendar.

There is one schedule that outlines all twenty of your study sessions on the following pages. You can also find this document to print inside of your file vault. Simply click the link to download and then print or save the document.

We recommend that your study time is limited to no more than 3-hrs per day in order to avoid fatigue and NCLEX burnout! This is a recommendation and you may find that you are able to handle more but I want you to be kind to yourself as we go through this process together! We believe that your content should be simple, easy to understand, and straight to the point because great nurses have compassion and know their content.

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USE REMAR V2 QBANK TO FOCUS ON CRITICAL CONTENT AREAS

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REMAR'S STUDY SESSION OUTLINE

This study calendar is designed to make your life easier! Each week do 5 study sessions. One study session each day is best to pace yourself. One study session a day also allows you time to process the information.

You will need to print the workbook in order for this calendar to be most effective. Remember to keep your study sessions under 3 hours per day. Check off each task once it has been completed. If you follow this calendar the course will be completed in 4 weeks.

STUDY SESSION #1

Watch / Complete	Read	Answer
☐ Pregnancy Overview – pg. 2 ☐ Normal & High-Risk Newborn – pg. 13 ☐ Infant Heart Defects – pg. 16 ☐ Pediatric Milestones – pg. 18	☐ Download & Print this Digital Workbook	☐ Clinical Subject Exam: Pregnancy

STUDY SESSION #2

Watch	Read	Answer
☐ Age Specific Nursing Care – pg. 21 ☐ Expected Changes in Aging – pg. 25 ☐ Diets – pg. 28	☐ Review Session #1 Notes	☐ Clinical Progress Exam 1 ☐ Clinical Subject Exam: Physiological Integrity

STUDY SESSION #3

Watch	Read	Answer
☐ Basic Care & Comfort – pg. 31 ☐ Orthopedics – pg. 35	☐ Review Session #2 Notes	☐ Clinical Pharmacology Exam 1 ☐ Clinical Progress Exam 2 ☐ Clinical Pharmacology Exam 2

STUDY SESSION #4

Watch	Read	Answer
☐ Medication Administration – pg. 40 ☐ Antibiotics – pg. 42	☐ Review Session #3 Notes	☐ Clinical Progress Exam 3

STUDY SESSION #5

Watch	Read	Answer
☐ Intravenous (IV) Therapy – pg. 43 ☐ Total Parenteral Nutrition – pg. 45 ☐ Pain Management – pg. 46	☐ Review Session #4 Notes	☐ Clinical Pharmacology Exam 3

STUDY SESSION #6

Watch	Read	Answer
☐ Substance Abuse – pg. 47 ☐ IV fluids – pg. 49 ☐ Clinical Math – pg. 50 ☐ Easy NCLEX Labs – pg. 55	☐ Review Session #5 Notes	☐ Clinical Progress Exam 4

STUDY SESSION #7

Watch	Read	Answer
☐ Easy Electrolytes – pg. 57 ☐ Diabetes Insipidus – pg. 58	☐ Review Session #6 Notes ☐ Quick Facts (QF) for NCLEX book pages 1-10 ☐ QF PHARMACOLOGY SECTION- Allergy medications, Analgesics, Antibiotics, Anticoagulants	☐ Clinical Pharmacology Exam 4* ☐ Clinical Progress Exam 5

STUDY SESSION #8

Watch	Read	Answer
☐ Positions – pg. 59 ☐ Disaster Management – pg. 61 ☐ Herbal Management – pg. 62	☐ Review Session #7 Notes ☐ QF PHARMACOLOGY- Anticonvulsants, Antidotes, Antineoplastic, Anti-Parkinsons	☐ Enter ReMar's Q-bank and create quizzes spend 30-45 minutes answering questions. Choose any subject you like.

STUDY SESSION #9

Watch	Read	Answer
☐ Blood Gas Interpretation – pg. 63 ☐ Chest Tubes – pg. 64 ☐ Vent Alarms – pg. 67 ☐ Congestive Heart Failure – pg. 68	☐ Review Session #8 Notes ☐ QF Pages 11-20 ☐ QF PHARMACOLOGY- ARBS, Ace Inhibitors, Antiarrhythmics	☐ Clinical Subject Exam: Arterial Blood Gas ☐ Clinical Progress Exam 6

STUDY SESSION #10

Watch	Read	Answer
☐ Diagnostic Procedures – pg. 70 ☐ Lowering Cholesterol – pg. 72 ☐ Ear Spotlight – pg. 73	☐ Review Session #9 Notes ☐ QF REVIEW-Any previous areas	☐ Enter ReMar's Q-bank and create quizzes spend 30-45 minutes answering questions

STUDY SESSION #11

Watch	Read	Answer
☐ Diabetes Overview – pg. 74 ☐ Endocrine Review – pg. 81	☐ Review Session #10 Notes ☐ QF pages 21-30 ☐ QF PHARMACOLOGY-Beta Blockers, Calcium Channel Blockers	☐ Clinical Progress Exam 7

STUDY SESSION #12

Watch	Read	Answer
☐ Therapeutic Communication – pg. 83 ☐ Psychological Concepts – pg. 84 ☐ Psychiatric Medications – pg. 89	☐ Review Session #11 Notes ☐ QF Pages 31-40 ☐ QF PHARMACOLOGY-Digoxin, Nitrates, Diuretics	☐ Clinical Progress Exam 8

STUDY SESSION #13

Watch	Read	Answer
☐ ECG Overview – pg. 94 ☐ Isolation Precautions – pg. 100 ☐ Accident & Error Prevention – pg. 103	☐ Review Session#12 Notes ☐ QF PHARMACOLOGY-GI Medications, Insulins	☐ Clinical Subject Exam: Therapeutic Communication ☐ Clinical Subject Exam: Safety & Infection Control

STUDY SESSION #14

Watch	Read	Answer
☐ Management of Care – pg. 105 ☐ Legal Issues in Nursing – pg. 105	☐ Review Session #13 Notes ☐ QF Pages 41-50 ☐ QF PHARMACOLOGY-Oral Antidiabetics, Natural Alternatives, Maternity	☐ ReMar Question Bank (up to 60 minutes only)

STUDY SESSION #15

Watch	Read	Answer
☐ Delegation & Assignment – pg. 107 ☐ Prioritization – pg. 108	☐ Review Session #14 Notes ☐ QF Pages 51-60 ☐ QF PHARMACOLOGY-Respiratory, Benzodiazepines, SSRIs, MAOIs	☐ Clinical Subject Exam: Prioritization ☐ Clinical Progress Exam 9

Hey guess what! You're in your final stretch of your NCLEX content review! How do you feel? At this point you should definitely have a test date scheduled. If you don't have your date yet, it's ok. I want to remind you that you can do this. Continue to study. IF you have more time before you test you can review the content again, finishing your final exams, or beginning your ReMar V2 Qbank assessment test.

STUDY SESSION #16

Watch	Read	Answer
☐ Tips to Master NCLEX	☐ Review Session #15 Notes ☐ QF PHARMACOLOGY-Phenothiazines, Atypical Psych drugs, TV drugs	☐ ReMar Question Bank (up to 60 minutes only)

STUDY SESSION #17

Watch	Read	Answer
☐ No videos to watch ☐ (Focus on course finals)	☐ Review Session #16 Notes ☐ QF Pages 61-66 ☐ QF PHARMACOLOGY-Easy Lab Values, Math	☐ Course Final Examination 1

STUDY SESSION #18

Watch	Read	Answer
☐ Review previous lectures from weak areas or start watching the lectures again from the beginning depending when you test.	☐ Review Session #17 Notes ☐ QF Pages 67-73 ☐ Clinical Skills-Section 1, Section 2, Section 3	☐ Course Final Examination 2

STUDY SESSION #19

Watch	Read	Answer
☐ Review lectures from weak areas	☐ Review Session #18 Notes ☐ QF Practice Exam ☐ Clinical Section 4, Section 5	☐ ReMar Question Bank ☐ (up to 60 minutes only)

STUDY SESSION #20

Watch	Read	Answer
☐ Review lectures from weak areas	☐ Review Quick Facts information that you have not memorized.	☐ ReMar Question Bank ☐ (up to 60 minutes only)

Before we start I need you to do me a favor – take a moment to clear your head, steady your thoughts, say a prayer or do whatever you need to do to mark THIS moment in time.

I know that you have the ability to be an amazing nurse so I need you to be committed to this process of learning the content that you need to get your nursing license in the next six to eight weeks! If you're ready to go repeat after me, and say...

"I CAN, I WILL, I MUST, PASS NCLEX!"

Now lets' get started!

Regina M. Callion

PREGNANCY

A.) Human chorionic gonadotropin(HCG) is the hormone responsible for pregnancy.

B.) Probable Signs

--

C.) Positive Signs

--

D.)

--

NCLEX Pro Tip: If LMP falls between January to March Add 7 Days to LMP then add 9 months

PREGNANCY

E. Monthly Doctor's Visits and Milestone Markers

Months	Doctor's Visits & Milestone Markers
Up to 28 weeks	

1.	
2.	

Critical Thinking Question: A woman has 4 children (2 singles and a set of twins) what is her parity?

PREGNANCY

F. Diagnostic Procedures:

1.

Indication	1. 2.
Administration	Invasive Procedure:
Client Teaching	4 NCLEX Teaching Points for Registered Nurses 1. 2. 3. 4. Watch for:

PREGNANCY

2.

Indication	
Administration	When to give?
Client Education	Indirect Coombs Test

3.

Indication	1. 2. 3.
Administration	Results- Reactive is negative= Non-reactive=
Client Education	

G. 6 Points of Client Education:

1. Morning Sickness	
2.	
3.	
4.	
5.	
6.	

H. Danger signs in pregnancy

--

If danger signs are present never assess client _____.

COMPLICATIONS OF PREGNANCY

1. Preterm Labor

Medications to stop premature labor

If you Give: _____ Watch for: _____

If you Give: _____ Watch for: _____

A.)	
B.)	
C.)	

If you give: _____ Watch for: _____

If you Give: _____ Watch for: _____

2. P _____ - _____

The three defining characteristics are:

--

Risk Factors

--

NCLEX Maternal
Emergency

Nursing Care:

Treatment: Only cure is to _____ the _____.

LABOR & DELIVERY OVERVIEW

J. L _____

1. Want to help labor along?
2. When you give oxytocin when do you Stop it?

K. Stages of Labor

1. First

Stages	Description
A.	Days before
B.	
C.	
D.	

NCLEX Pharmacology Tips: Pain Management During Labor

Medication	Notes
1. _____ Blocks	
2. Hydromorphone	See Quick Facts
3. Morphine	See Quick Facts

2. Second Stage:

3. Third Stage:

4. Recovery Stage:

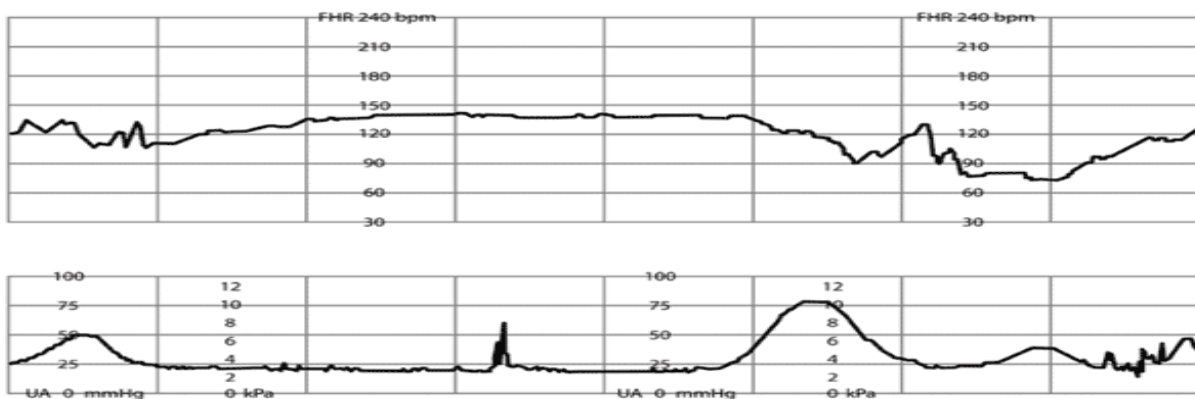
NCLEX Advanced Topics:

_____ helps to tell the _____

Reactive strip = _____

A reactive strip contains _____:

- A.
- B.
- C. Long term variability



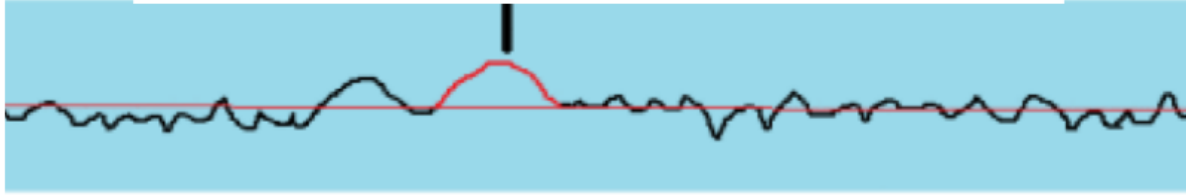
FETAL HEART STRIP ORIENTATION

Top box represents:

Bottom box describes:

LABOR & DELIVERY OVERVIEW

1. Accelerations

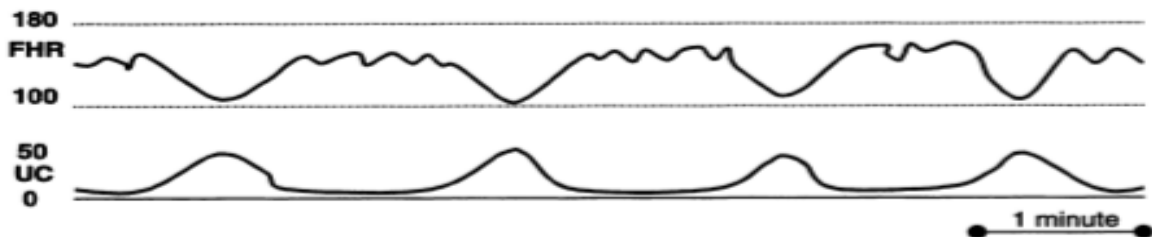


Accelerations are an _____ in the baseline heart rate.

Good sign	
Cause	Fetal

2. Decelerations

1. Fetal Heart Reading:

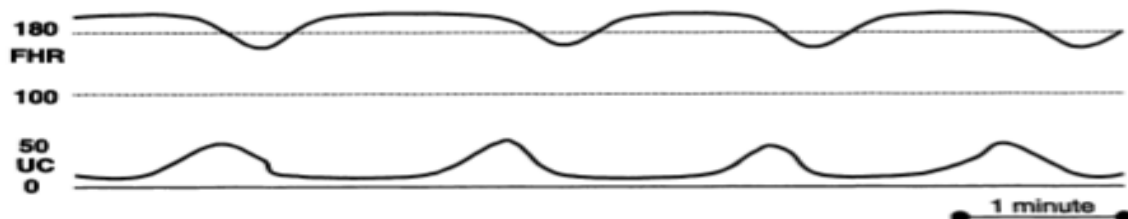


Caused by: _____

Looks like: _____

Treatment: _____

2. Fetal Heart Reading:

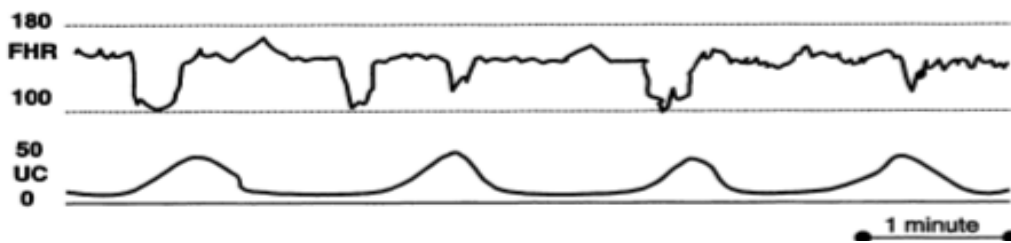


Caused by: Problem with the _____

Looks like: Heart rate _____

Treatment: _____

3. Fetal Heart Reading: Variable



Caused by: _____

Looks like: _____

Treatment: _____

LABOR & DELIVERY OVERVIEW

Post-partum Assessment

Biggest risk for post-partum complications is within:

<u>B</u>
<u>U</u> terus Uterine atony
<u>B</u>
<u>B</u>
<u>L</u> Color: 1. 2. 3.
<u>E</u>
<u>H</u>
<u>E</u> xtremities- assess lower extremities for edema, tenderness, varices, increased skin temperature

LABOR & DELIVERY OVERVIEW

Client Education Point	
1.	2. Client teaching: 1. 2. 3. 4. Notify healthcare provider if heavy _____ starts to occur.

NORMAL & HIGH RISK NEWBORN

1. Apgar Scoring is done at _____ and _____ minutes.

Sign	Score = 0	Score=1	Score=2

Score at _____ minutes is more valuable.

Eyes: _____

Temperature: _____

Pulse: _____

Respirations: _____

Abdomen: _____

_____ Care
Allow _____ to _____.
By _____ cord should have _____.
Monitor for signs

1. _____ Addicted Newborn

Symptoms:

What is the best way to test for illegal drugs in infants? _____

Nursing care:

Cluster Care Examples:

2.

--

3. Fetal

Characteristics to know

1.

2.

3.

4.

Risk for:

Treatment:

4.

5.

Treatment:

Client Education for Parents:

NCLEX Tip: Nutrition is a major concern for these birth defects.

INFANT HEART DEFECTS

B_____ Baby	P_____ Baby
1.	1.
2.	2.
3.	Hole between 2 upper chambers
4.	3. Patent Ductus Ateriosus-

All Blue Baby problems begin with the letter _____.

Emergency Position:

Most Important Infant Heart Defect

Tetralogy

Mnemonic	Clinical Condition
R	Right Ventricular Hypertrophy
O	
P	Pulmonary
V	

INFANT HEART DEFECTS

How should a nurse identify a child?

Critical Thinking Questions

1. An infant with Tetralogy of Fallot is discharged with a prescription for digoxin elixir. The nurse should instruct the mother to:

1. Administer the medication using a nipple.
2. Administer the medication using the calibrated dropper in the bottle.
3. Administer the medication using a plastic baby spoon.
4. Administer the medication in a baby bottle with 1. Oz of water.

2. A nurse is caring for an infant diagnosed with Tetralogy of Fallot which of the following are expected findings? Select all that apply.

1. Clubbing of the fingers
2. Dyspnea with activity
3. Hypoglycemia
4. Fever
5. Cyanosis

3. Calculate the Apgar Score for each category and report the total.

Baby Jones	Score
Heart rate = 85	
Respirations= Full and normal	
Muscle= No spontaneous movements noted	
Appearance= Arms & legs blue	
Reflex= No response	
Total	

PEDIATRIC DEVELOPMENTAL MILESTONES

AGE	ACTIVITY	NCLEX TIPS
S _____	Erikson's Stages of Psychosocial development: Freud Stages of Psychosexual development:	Best Toy
_____ months O _____ P _____		First Solid food?
_____ months		Best toy:

_____ years old P _____ _____	Erikson's Stages of Psychosocial development: Identity vs. Role Confusion 13-21 years old Freud Stages of Psychosexual development: Genital 12-20 years old	
--	---	--

Children's

	No sense of _____.
	_____ expressed towards

NCLEX PRACTICE QUESTIONS

1. A registered nurse is teaching a conference on the normal development of four-year-olds.. Which of the following statements should be included? Select all that apply.

1. "The child may exhibit a fear of the dark"
2. "The normal weight should double during this age."
3. "According to Erickson's Stages of Psychosocial development the child is experiencing industry vs. inferiority."
4. "Children at this age understand death to be permanent."

2. A teenager is admitted to the hospital with a diagnosis influenza. The teenager refuses to let his friends come to visit. The nurse should know this is a result of:

1. His inability to explain what is happening to the friends.
2. His perception of altered body image.
3. His need to be in the center of attention.
4. His anger on being left out of school activities.

3. The nurse is caring for an 11-month-old admitted to the hospital with a respiratory infection. The child is in a croup tent with supplemental oxygen. Which toy is **most** appropriate for the nurse to recommend to the parents?

1. A push-pull toy.
2. A toy car.
3. A soft multi-colored ring stacking set.
4. A wind-up jack in the box on wheels.

AGE SPECIFIC NURSING CARE

No matter the age all clients have the same rights for:

- 1.
- 2.
- 3.
- 4.
- 5.
- 6.
- 7.

Age Group:

The top 2 nursing concerns are:

- 1.

Pediatric patients are 3 times more likely to have a medication error.

- 2.

Before administering a medication ask client for:

Critical Thinking Question: If a patient is too young, can the parents answer? _____

NCLEX Teaching Point:

Patients have to be positioned properly before giving medications or feedings to prevent

_____.

Separation from the primary care giver is:

Adults number one fear is:

ReMar Nurse Study Tip:

Age Group:

Goals:

- Develop _____ with the opposite _____.
- Establishing a _____.
- Coping with _____.

They have the need to establish independence from primary care giver.

Medication	NCLEX Points

Psych Priority:

1.

Age Group:

1. Resolve _____

2. When sick rearranging responsibilities of work, child rearing etc.

Age Group:

#1 Concern is to:

Community Health Nursing

There are 3 levels of prevention:

Level of P _____	Techniques
1. The Goal:	
2. The Focus:	
3. The Focus:	

AGE SPECIFIC NURSING CARE

Psych concerns:

Caring for _____ & _____ at the same time.

Age Group 60-and over

Goals

Positively _____ the loss of a _____.

Transition into _____.

Maintain physical _____ and prevent cognitive _____.

NCLEX Nursing concerns

1.	Lower metabolism Higher body fat
2.	Prevention of Falls #1 thing to give: Non slip socks, call light in reach, bowel/bladder program where you are able to anticipate ambulation needs
3.	1. What is the most important lab value to determine the nutritional status of a client? 2. 3.

EXPECTED CHANGES DURING AGING

S _____	Changes Tested on NCLEX
1.	Peripheral circulation:
2.	
3.	
4. Reproductive System	

5.	Decreased range Decreased _____ .
6.	
7.	
8.	Decreased activity in CNS & PNS Decrease in _____ time and reflex times. Mental function should _____ .

9.	_____ in the _____ of immune system. Encourage:
10.	Decrease secretion of: Decrease in: Will a decreased insulin production make the blood sugar increased or decreased?
11.	Loss of:

--

DIETS

Diet	Indication	Food
	1. Or 2.	
	1.	
	1.	
Pureed	1. 2.	How should the foods be placed when feeding?

	1.	Any food that can be easily broken down
	1.	Cannot have: Avoid CAP
Protein _____	1.	Avoid: Why should renal clients avoid protein?

Restricted	1. 82 grams or 1 gram heart healthy	
	1.	
	1.	Avoid Purine
	1. 2.	
	1.	
	1.	Bread? Spaghetti? Pie? Cookies? Waffles? Pancakes?

BASIC CARE & COMFORT

A. Hygiene

When bathing clients always start with

1. Elderly Care	The Skin Is:
2. _____ _____ Care	_____ and dry feet daily Do not Do not cut: Do not use _____ between the toes Check for:

Who should be delegated to give the client a bath?

B. Rest

Adequate sleep supports _____.

Age	NCLEX Notes on

C.

To know hydration you have to be able to properly assess dehydration.

--

In dehydration urine levels may drop below the normal:

--

Treatment: Rehydration

ReMar's Tip: Oral hydration can be just as effective as IV hydration if started early enough.
--

NCLEX TIPS:

- 1.
- 2.
- 3.

D. Bladder & Bowel Elimination

E. Urine

How much urine is produced each day?	
Specific gravity	1.016 - 1.022

Critical Thinking Question: Why are UTIs more common in women than men?
Alteration in normal urine pattern matching

Anuria	A. glucose in the urine
Glycosuria	B. involuntary urination at night
Hematuria	C. no kidney function
Pyuria	D. blood in the urine
Enuresis	E. Pus in urine

NEED TO KNOW NCLEX SKILL: _____

Start with an _____.

Ask patient to void then: throw away.

All _____ must be kept in _____ container.

If one urine sample missed:

Keep _____ on _____.

Critical Thinking Question

Which of the following is the correct order for the registered nurse to perform an abdominal assessment?

1. Auscultation, percussion, inspection, and palpation.
2. Inspection, auscultation, percussion, and palpation.
3. Inspection, palpation, percussion, and auscultation.
4. Percussion, inspection, auscultation, palpation.

F. Bowel

Handling Normal Stool	Which Precaution:
------------------------------	--------------------------

Factors Affecting Bowel Patterns	
1.	
2.	
3.	
4.	
5.	

BASIC CARE & COMFORT

Definition of constipation	
Definition of diarrhea	

Bowel Tests to know

1.	Screens for:
2.	

NCLEX Skills Question:

The nurse administering an enema to a client knows that the tip of the tubing should be inserted into the rectum while the client is in a sitting position, as on the toilet. True or false?

A. True

B. False

ORTHOPEDICS

1. Canes The cane moves with the _____
then the _____ follows.

2. Casts

Use _____ of our hands to handle during first 24 hours.

Do not use _____.

Do not get the cast _____.

What about scratching underneath cast?



Always remember to do _____.

NCLEX Emergency:	This is an: Assess for 6 P's Nursing Interventions:
--------------------------------	--

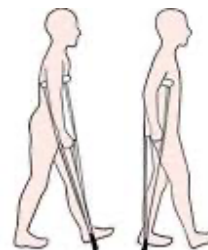
ORTHOPEDICS

3. Crutches-

Measurements need to be:

Top of crutches should be _____ below _____

The handgrips should be:



Gaits

Three Point Gait

2 Point	Move left crutch with right foot then right crutch with left foot.
3 Point	Move crutches and weaker leg, then strong leg.
4 Point	Move left crutch, then right foot, then right crutch and then follow with the left foot.
Swing Through	Move both crutches forward then bear all weight on crutches and swing legs forward at the same time.



Stair walking with Crutches

Going up	
Going down	

4. Walkers-



Never
Elbows
Step
Do not pick:

Promoting Circulation	1. Need _____
Thromboembolic Compression Stockings (TED)	A. Put them on while client is _____. Time Limit:
2. (SCD)	Monitor for:

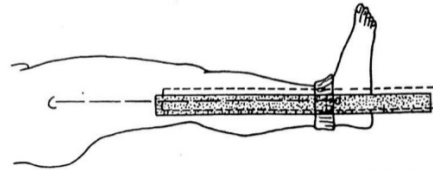
Clients are not allowed to:

- a. Cross
- b.
- c.

Advanced Clinical Topic:	_____ is a _____ _____ to limbs, bones, or tissue.
--------------------------	---

ORTHOPEDICS

There are 2 types of _____:



1. _____

Indications: 1. Femoral fractures 2. 3.	The _____ is applied over a _____. _____ are used to exert a pulling force. Examples: Heels should be: Time Limit:
--	---

2. _____

Indications: 1. Fractures 2. Fractures	Metal pins or wires surgically applied.
--	--

Items

- A.
- B.
- C.
- D.

Client's Activity Level:**Watch for :**

- 1.
- 2.
- 3.
- 4.

Critical Thinking Questions:

1. A client was hit by a motor vehicle 12 hours ago and is being discharged with a Plaster of Paris cast of the right leg. Which of the following statements need follow-up teaching?

1. "I will not scratch the skin under my cast."
2. "I will use the palms of my hands to handle the cast for the next 8 hours."
3. "I will not get my leg cast wet during basin baths."
4. "I will notify the healthcare provider if I feel numbness in my leg."

2. A nurse is caring for a client in skeletal traction. Which of the following are expected findings? Select all that apply.

1. Redness and inflammation at the pin site.
2. Purulent drainage at the pin site.
3. Serous drainage at the pin site.
4. Chest pain due to immobilization.
5. Loosening of the pin with frequent movement.

3. A nurse is caring for a client with signs of acute compartment syndrome. The client is reporting numbness and tingling in the left lower extremity. Which is the **priority** action of the registered nurse?

1. Notify the healthcare provider.
2. Obtain baseline vital signs.
3. Assess respiratory status.
4. Assess pedal pulses.

MEDICATION ADMINISTRATION

Before you give medications check the rights there are many.

- | | | | |
|------------|-------------------|--------------------------|----------|
| 1. Patient | 2. Drug | 3. Dose | 4. Route |
| 5. Time | 6. Documentation. | 7. Allergies & Many more | |

Verify

1. PO	PO-
2.	
3.	
4.	

5.	
6.	
7.	

- 1.
- 2.

ANTIBIOTICS

	Examples	How They Help	How They Harm
1.	*Vancomycin is not an aminoglycoside but often added because of the similar side effects		
2.			
3.			

ANTIBIOTICS

Clinical Judgement Practice Questions

1. A client is scheduled to receive clindamycin at 9:00 am, at which time should the trough level be drawn by the nurse?

1. 07:30
2. 08:00
3. 08:45
4. 09:00

2. A teenage client has been prescribed a tetracycline for moderate acne. Which of the following statements is the highest priority in the client education about the medication?

1. "Use sunscreen when you are exposed to direct sunlight."
2. "Monitor the teeth for color changes."
3. "Report any signs of hearing loss to the healthcare provider."
4. "Reduce the amount of fat in your diet to decrease the presence of ketones."

3. A nurse is caring for an elderly client who has pneumonia. The healthcare provider prescribes penicillin PO for 14 days. When the nurse asks the client if she has a penicillin allergy the client states she does not know as she has never taken the medication. Which of the following is the best response by the nurse?

1. "I will administer this medication and stay with you for monitoring after you take it."
2. "I will hold the medication and clarify the order with the healthcare provider."
3. "I will notify the healthcare provider and suggest a different antibiotic."
4. "I will notify the pharmacist and discuss other alternatives."

INTRAVENOUS (IV) THERAPY

Devices used for IV administration
1.
2.

IV COMPLICATIONS

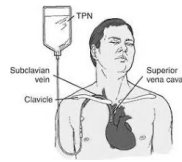
Complications	Signs	Nursing Interventions
1.		1. Stop Infusion Immediately 2. 3. Elevate extremity. 4. 5. Warm compress is more than 30 minutes ago. 6. 7. Restart the IV 8. Document
2.		1. Stop IV infusion. 2. 3. Elevate extremity 4. 5. Notify healthcare provider 6. 7. Document *Some references may say heat is also an acceptable treatment option.

NCLEX Emergency	Air _____
Signs	1. 2. 3. 4.
Nursing Interventions	1. 2. 3.

Critical Thinking Question:

1. A client presents to the emergency room after a hit and run accident. The client has sustained massive trauma and is in a hypotensive crisis situation. Which of the following intravenous cannulas will be **most** beneficial for fluid resuscitation?

1. 22 gauge
2. 20 gauge
3. 18 gauge
4. 14 gauge



TOTAL PARENTERAL NUTRITION

What are the nutrients going through?

Who needs TPN?

Examples

What labs to monitor?

What electrolyte imbalances can TPN cause?

What is your emergency substitute for TPN?

How do you stop TPN?

NCLEX Note: How often does the nurse change the tubing?

PAIN MANAGEMENT

Pain is _____. It can feel different from client to client.

Rating	Technique
Adults	
Babies/Children	

Non-verbal cues of pain

NCLEX notes about PCA Pumps.

Critical thinking question:

Should the nurse teach about PCA pump before or after surgery?

2 Things to Monitor
1.
2.

SUBSTANCE ABUSE

Substance Abuse	Continued use of a substance
-----------------	------------------------------

Substance abuse can be _____. An _____ is considered a _____.

Substances that are abused are:

A. Alcoholism-

Delirium Tremens:

- 1.
- 2.
- 3.
- 4.
- 5.

2 Other Symptoms to Know

1.	
2.	

SUBSTANCE ABUSE



Medical Emergency

Delirium Tremens Treatment:

Medicate with anti-anxiety medication such as:

1.

2.

or

_____ medications such as:

1.

Teaching point:

Clients must avoid:

NCLEX Pro Tip:

Medications that contain alcohol:

B. Opioid Addiction

Examples:

Room

Withdrawal

_____ : is a legal narcotic that can be used as a substitute.

IV FLUIDS

There are _____ of intravenous fluids.

NCLEX Pro Tip: All 3 are used to treat some form of _____.

IV Fluids	Examples
1. _____ Lower: 1. Osmotic Pressure 2. Concentration of	What type of dehydration does this treat? Also used for: Avoid:
2. _____ Same: 1. 2. Does not draw or push fluid into the cell.	Avoid clients with: 1. 2.
3. _____ Higher: 1. 2. Does not draw or push fluid into the cell. Avoid: <i>Clients with compromised renal functions</i> <i>Clients with congestive heart failure</i> <i>Clients with diabetic ketoacidosis</i>	What type of dehydration does this treat? Intravascular dehydration w/ cellular over hydration Used for:

General Nursing Intervention Tips:

CLINICAL MATH

Florence Nightingale:

Mother of modern nursing

NCLEX Units of Measurements

**There will be no complex math during this study session.
This math will be based off the Quick Facts for NCLEX book.**

A. Working with Numbers:

_____ describe _____.

Example: height, weight, blood pressure.

These numbers can be _____ or _____.

Calculating Fluid Balance

Formula:

Can a client's fluid balance be negative?

What does this mean?

A client's intake is 800 mL.

The same client's output is 2000 mL.

What is the net fluid balance of this client?

Critically think:

Which types of clients need negative fluid balances?

- 1.
- 2.
- 3.

Watch for these clients with negative fluid balances (not helpful)

4. Dementia, Alzheimer's

CLINICAL MATH

5.

B. Multiplication:

Let's Work!

Multiplying by _____.

79 x 0	
6 x 0	
144 x 0	

Multiplying by _____.

12.5 x 1	
4 x 1	
62 x 1	

Multiplying by _____.

12 x 10	
83 x 10	
95 x 10	

Multiplying by one _____.

10 x 100	
35 x 100	

Metric system uses multiples of _____.

C. Division:

In medical math division is represented by a _____.

$$12 \div 3 = 4 \quad \text{or} \quad \frac{12}{3} = 4$$

$$10 \div 2 = 5 \quad \text{or} \quad \frac{10}{2} = 5$$

$$24 \div 8 = 3 \quad \text{or} \quad \frac{24}{8} = 3$$

Real Life Practice:

1. A client is prescribed 750 mg of lansoprazole per day. The medication comes in 250 mg tablets. How many tablets will be administered per day?

Remember _____ just means a part of a _____.

$\frac{1}{4}$ 0.50 1:8 35%

These are all _____.

Reduce these fractions to the lowest terms:

1. $\frac{10}{20}$	
2. $\frac{5}{15}$	
3. $\frac{4}{32}$	

D. Conversions: _____ System

Metric system is easy because it uses _____ math.

mili	1/1000 or 0.001
centi	1/100 or 0.01
deci	1/10 or 0.1
Confusion Alert:	Another name for the metric system

E. System:

Weight	Liquids
	Cup Fluid oz. (fl oz.) Tablespoon (Tbsp) Teaspoon (Tsp.) Drop (gtt)

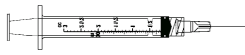
What about the apothecary system? Examples are listed here these will not be found on the NCLEX but you should be familiar with the name for historical reference.

Drams.	Scruple.	Grains.	Minim.
--------	----------	---------	--------

Most conversion problems require you to remember _____.

Metric Unit	Household Unit		
1 mL			
5 mL			
15 mL			
30 mL			
237 or 240 mL			
1 kg			

Record ice chips _____

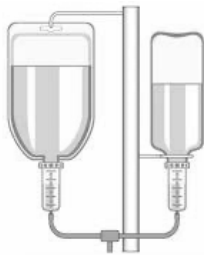


CLINICAL MATH PRACTICE QUESTIONS

CRITICALLY THINK

1. If a client took 6 oz. of ice chips, how many mLs of fluid should the nurse record?
2. A client is prescribed simethicone 30 mL PO BID. The medication comes in 50mg/500 mL. How many teaspoons are in each dose?
3. Which client is most appropriate to take a medication in the form of a liquid? Select all that apply.
 - 1-A. A 26-years-old with a compromised IV site.
 - 2-B. A 9-month-old with depressed fontanelles.
 - 3-C. A 9-years-old with celiac disease.
 - 4-D. A 43-years-old client newly diagnosed with stroke.
 - 5-F. A 63-years-old client on strict bed rest.

Dosage Calculations



2. A nurse is instructed to administer heparin with a weight-based protocol. The client weighs 220 lbs. She must give 80 units/kg bolus and then start an infusion at 18 units/kg/hr. The heparin comes in 25,000 units in 500 mL of 0.9 NS.

A. What is the client's weight in kilograms?

B. Calculate the bolus dose?

3. Calculate the flow rate?

Flow rate can also be called _____.

Infusions can be _____ or _____.

CLINICAL MATH

Simple Formula for Calculating Flow Rate

Flow rate =

Example: A client is to receive 500 mL of D5 0.45% NaCL over 24 hours. Calculate the IV flow rate.

So for our example using the formula we would write.

Step 1

Flow rate =

Simple Formula for Calculating Flow Rate

Flow rate = Total $\frac{\text{Volume}}{\text{Time}}$

3. A client is ordered 1200 mL D5W IV. The IV solution is to infuse over 10 hours by an infusion pump. Calculate the flow rate in milliliters per hour.

Conversion Factor: Minutes to Hours

Formula

4. A nurse is scheduled to administer ketamine 800 mL IV. The IV solution is to infuse over 240 minutes. Calculate the flow rate in milliliters per hour.

Simple formula for calculating the drip rate

Drip rate (gtt/min)=

5. A nurse is working in a portable hospital tent. She is working with a manual IV infusion system. Her client must receive 0.9 NS 1000 mL IV over 12 hours. The drop factor is 20 gtt/mL. Calculate the drip rate.

Simple Formula to Calculate Units per Hour

6. A client's heparin drip is infusing at 11 mL/hr. on an infusion pump. The premixed solution is 25,000 units of heparin in 250 mL D5W. What hourly dose of heparin is the client receiving?

Step 1.

Find out how many units are in 1 mL.

Heparin Units= units in mL
Volume (mL)

Step 2.

EASY NCLEX LABS

Labs	Purpose	Values
Hemoglobin Hgb	Transport oxygen to tissue and Co2 back to lungs. RBC's are made up of hemoglobin. If this is Low think Iron deficiency anemia	-male 14-16.5 g/dl -female 12-15 g/dl
Red blood cells RBC	These carry oxygen from the lungs to the tissues around your body.	-male 4.5-6.2 % -female 4-5.5 %
Hematocrit	The hematocrit is the ratio of the volume of packed red blood cells to the total blood volume. If it's Low than that indicates a decrease in O2 capacity.	-male 41-51% -female 36-46%
White Blood Cells WBC	This is the body's defense against infectious organisms and foreign substances. Chemotherapy would make this laboratory value decreased.	5,000-10,000 /uL or mm3
K	Potassium is a mineral & electrolyte Carries an electrical charge important for many cardiac functions.	3.5-5.1 mEq/L
Na	Sodium maintains fluid levels, muscle and nerve functions.	135-145 mEq/L
Ca	Calcium is a mineral and electrolyte that is important in bone and teeth development	8.6-10 mEq/L
Mg	Magnesium is important for nerves and muscles. It helps neutralize acid in the stomach An example of a medication that contains magnesium is Milk of magnesia (laxative)	1.6-2.6 mEq/L
Cl	Chloride keeps the fluid balance in/out of cells in check. We get most of chloride from the sodium chloride we eat. It is digested in the intestines.	95-105 mEq/L
CO2	Carbon dioxide is a waste product of the respiratory system but also is necessary for our body to function properly.	22-32 mEq/L

BUN Blood urea nitrogen Liver/kidney test	The test measures the level of urea in the blood. Urea is a waste product that comes from protein. It's what is left over. Urea is made in the liver but then it is the kidneys' responsibility to get rid of it through the urine.	8-25 mg/dL (microgram/deciliter)
Creatinine	This is a test of kidney function. The creatinine clearance rate helps to estimate the glomerular filtration rate (GFR)	0.6-1.3 mg/dL High creatinine signals renal failure
Liver Enzymes	1. aspartate aminotransferase (AST or SGOT) enzyme that is released when the liver or muscle cells are injured. 2. alanine aminotransferase (ALT) (Sgpt) like AST this is an enzyme that is found in the liver that is normally low but when liver damage has occurred this value will be elevated.	1. 10 to 40 IU/L. 2. 5-35 U/L These values elevate with hepatitis or jaundice.
Glucose	Serum glucose is the amount of sugar in the blood. Most of our sugar comes from carbohydrates.	70-110 mg/dL
PTT Partial Thromboplastin Time	A blood test that helps doctors assess the body's ability to form a blood clot. The test measures how many seconds it takes for a clot to form.	Clotting should occur in 60-70 seconds If the client is taking an anticoagulant it will be 1.5 x 2.5 times longer 120 to 140 seconds.
aPTT Activated partial thromboplastin time	A blood test that is also used to measure the time it takes for a clot to form however an activator is added to the blood that speeds up the clotting time. This test is used mostly to monitor a client's response to heparin.	30 to 40 seconds If the client is taking an anticoagulant it will be 1.5 x 2.5 times longer 60 to 80 seconds.
INR international normalized ratio	This is a blood test to determine the clotting time when a client is on an anticoagulant usually vitamin K antagonists such as warfarin.	1-2 2.0 to 3.0 is standard for clients taking warfarin.
Urine specific gravity	This is done to test kidney function. This test looks at all of the particles in the urine.	1.016-1.022 Urine sample collection 1 to 2 oz. first thing in the am.
Platelets	Smallest of the 3 major types of blood cells. Which are red, white, and platelets. The function is to prevent bleeding.	150,000-400,000 μ L microliter

EASY ELECTROLYTES

Electrolyte	Hyper Signs	Hypo Signs
1.	*Again please note the generic name is sodium polystyrene.	
2.		
3.		

4.		
5.		

DIABETES INSIPIDUS VS. SYNDROME OF INAPPROPRIATE ANTIDIURETIC HORMONE SECRETION

Both

Diabetes Insipidus

1. Diabetes Insipidus is too little _____.

Signs

Treatment:

Syndrome of Inappropriate ADH

2. _____-is too much _____.

Signs

Treatment:

POSITIONS

1.	4 types of Fowler's Low-_____ degrees Semi- _____ degrees Standard-_____ degrees High-_____ degrees Low fowler's: Semi: Standard: High:
2.	This position is for:
3.	What is this client at risk for due to being in this position for a long time?
4.	

5.	
6.	
7.	

DISASTER MANAGEMENT

_____ Disasters	_____ Disasters
Client priority is based on:	Client priority use: A- B- C-

RN Task what does it mean to triage:_____

Disaster Management. _____ *Coded System of* _____

	Bodily injuries but the client is stable.

Discharge Rules

Use the other ABCs	Who goes home or get relocated to make room for new clients 1. 2. 3.
--------------------	---

HERBAL MEDICATIONS

Herbal	Action	Client Teaching
1. St. John's Wort Psychiatric Medication		
2.		
3.		
4.		
5.		
6.		
7.		
8.		

General Client Teaching:

BLOOD GAS INTERPRETATION BY NUMBERS

pH	HCO ₃
Below 7.35=	
Above 7.45 =	

THINK R.O.M.E.! Respiratory Opposite Metabolic Equal

A.) How would you interpret this blood gas? pH 7.53 PaCO₂ 33 HCO₃ 33 PaO₂ 72

pH  7.53 HCO₃ 33 = _____

B.) How would you interpret this blood gas? pH 7.10 PaCO₂ 24 HCO₃ 45 BE 3

pH 7.10 _____ HCO₃ 45 _____ = _____

C.) How would you interpret this blood gas? pH 7.32 PaCO₂ 35 HCO₃ 17 PaO₂ 89

pH 7.32 _____ HCO₃ 17 _____ = _____

BLOOD GAS INTERPRETATION BY DIAGNOSIS

1st question:

Y	N
R	M
Respiratory Alkalosis=	Metabolic Alkalosis=
Respiratory Acidosis=	Metabolic Acidosis=

Critically Think:

- 1) Which blood gas value would you expect to see in a client with a pulmonary embolus
- 2) Which blood gas value would you expect to see in a client who has diabetes mellitus type 2?
- 3) Which blood gas value would you expect to see in a client who has chronic obstructive pulmonary disorder?
- 4) Which blood gas value would you expect to see in a client who has a pancreatic fistula and diarrhea?
- 5) Which blood gas value would you expect to see in a client 26 weeks pregnant with hyperemesis gravidarum?

*For NCLEX you do not have to worry about compensated or partially compensated blood gas interpretation.

CHEST TUBES

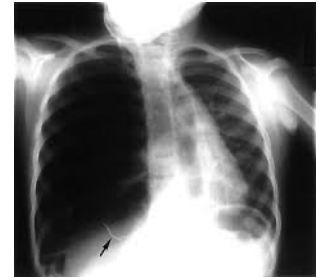
A.) Chest tube is a _____ drain that allow fluids or air to escape the pleural space

Remember normal breathing works on _____.

Negative pressure is the idea that when humans inhale it is a result of the diaphragm contracting and moving down and the rib muscles move out. This causes the lungs to expand. The pressure inside the lungs drops. And it is the negative pressure that sucks the air in.

Chest tubes are needed whenever the _____ in the pleural space is disrupted.

Tension pneumothorax- _____ is in between _____ and _____ which can be caused by trauma, surgery, falls etc. Outside air creates a one way valve inside the lung.



Classic signs of a tension pneumothorax:

Trachea deviation	Yes

This is a medical emergency, client needs treatment right away!

Treatment of tension pneumothorax:

CHEST TUBES

B.) Chest tube Setup: All chest tube systems have these three chambers

Collection chamber	Water Seal	Suction Control
Purpose is to:	Purpose is to:	

C.) Care of a client with a chest tube

- 1) Assess client for:
 - 2) Chest tubes should be placed _____ chest level.
 - 3) Do not milk or strip chest tube without a doctor's order.
 - 4) Daily _____ x-rays are needed to check _____.
 - 5) Clients will have an _____ dressing at the insertion site.
 - 6) Never clamp a chest tube without a M.D. order.
- D.) Common NCLEX troubleshooting.....

1. Noticed the water seal is broken

A. Place the distal end of the tube in 2 cm of sterile water.

2. Pulled the chest tube out

A. Use a _____.

B. Cover the opening with an _____.

C. _____ the dressing on _____ sides only so that you can allow:

Critical Think: What is the difference between a regular sterile dressing & occlusive dressing?

3. Complaints of pain won't comply-medicated and have the client to cough and deep breath
- A.
- B.

For NCLEX exam, what should you always have at the bedside of a client with a chest tube.

- 1.
- 2.
- 3.

Critically Think Future Nurses

1. What kind of lung sounds would the nurse expect to hear with a client who needs a chest tube? Select all that apply.

1. Wheezes
2. Crackles
3. Stridor
4. Diminished
5. Pleural friction rub

2. When caring for a client with a chest tube what should the nurse do to evaluate the effectiveness of the chest tube?

1. Empty chest tube drainage every shift
2. Mark chest tube drainage every shift
3. Clamp the chest tube when transferring the patient
4. Add water to the water seal chamber when she notices it is low

3. What should be done once the fluid in the water seal chamber no longer fluctuates with inspiration or expiration?

4. After a client has his chest tube removed by the healthcare provider which dressing should the nurse have ready to place over the incision site?

1. Transparent film dressing
2. Xeroform petroleum dressing
3. Seaweed healing dressing
4. Sterile cotton dressing

VENT ALARMS

ReMar Tip about Ventilator Alarms	High alarm sound= Caused by: mucous, blockage, biting
	Low alarm sound= Caused by:
If you don't know what to do then:	Disconnect the client and manually resuscitate them.

H.O.L.D. Help

H	High Alarm
O	Obstruction
L	Low Alarm
D	Disconnection

CONGESTIVE HEART FAILURE

When the _____ cannot pump enough blood and nutrients to meet the needs of the organs.

Big Problem:

_____ Side	_____ Side
L think	R think
Signs:	Signs:

Most clients will have failure _____ .

B.) Diagnostic Tests

C.) Medications

- 1.
- 2.
- 3.
- 4.

D.) Nursing Interventions

CONGESTIVE HEART FAILURE PRACTICE QUESTIONS

1. Mr. Green is scheduled to receive furosemide 60 mg IV BID for a diagnosis of congestive heart failure. The medication will have which of the following effects? Select all that apply.

1. Decrease blood pressure
2. Increase urine output
3. Increase blood pressure
4. Decrease urine output
5. Decrease pain
6. Increase edema

2. A 62-years-old client presents with dyspnea and blue colored nails and lips. The client has a suspected history of congestive heart failure and is admitted to emergency room. The client has not been compliant with his medication regimen and states he has not taken his hydrochlorothiazide for 4 days. The nurse should anticipate a diagnosis of which of the following?

1. Pneumonia
2. Pulmonary edema
3. Pneumothorax
4. Atelectasis

3. You are teaching the parents of a child with congestive heart failure about fluid intake. Which statement indicates understanding of monitoring fluid retention?

1. I will calculate all of the fluids that my child drinks as this is the best method to monitor fluid retention.
2. I will weigh each diaper daily as this is the best way to monitor fluid retention.
3. I will weigh the child each day at the same time as this is the best way to determine fluid retention.
4. I will listen to the lungs with my stethoscope as this is the best way to monitor fluid retention.

4. A client comes into the wellness clinic after being diagnosed with congestive heart failure. She complains of becoming tired after only very little activity. Which activity suggestion would the nurse give to preserve energy and decrease oxygen demands?

1. Setting a specific time during the day and accomplishing all daily tasks at one time.
2. Eating small frequent meals throughout the day.
3. Removing oxygen therapy during rest to build up a tolerance without it.
4. Exercise shortly after waking up in the morning when energy levels are highest

DIAGNOSTIC PROCEDURES

1. Position: Client Teaching:
2. _____ non-invasive test that uses _____ to create a detailed picture. Position: Client Teaching:
3. Position: Client teaching:
4. Position: Client Teaching:
5. Position: Client teaching: <i>Before exam</i> <i>After exam:</i>

DIAGNOSTIC PROCEDURES

6.

Position:

Client Teaching:

7. Angiogram or Arteriogram (RN only topic)

Position:

Client education:

Before Exam:

Medications to hold: Metformin, anticoagulants

After the exam:

1. Assess the:

2. Bedrest for _____ to _____ hours.

**Note: Some references say hold Metformin 24-48 hours before diagnostic study requiring IV iodine contrast media.*

LOWERING CHOLESTEROL

The goal of therapy is to lower _____ and _____.

Why does cholesterol matter?

Values to know:

- LDL(Bad) =
- HDL (Good)=
- Total Cholesterol=
- Triglycerides=

Examples of Dyslipidemias:

- *Simvastatin*
- *Rosuvastatin*
- *Atorvastatin*

Note: Drug name ends in: Statin

Side effects of Statins:

What about the B3 vitamin *Niacin*?

Side effects of Niacin:

Tip: Avoid flushed face by giving aspirin 30 minutes before treatment.

When your client is on a dyslipidemia assess them for?

What are the three types: _____.

Which organ is damaged due to free flowing muscle fibers?

How will the muscle tissue be excreted?

How to treat:

NCLEX teaching about lowering cholesterol:

Goal of Low Cholesterol diet:

Avoid:

Dairy foods such as cheese, butter, ice cream, egg yolk

Foods to include: avocados- which help raise HDL and lower LDL

EAR SPOTLIGHT

Meniere's Disease: A _____ disease that occurs in the _____ ear resulting in too much endolymphatic _____.

Does Meniere's affect 1 ear or 2 ears? _____.

The cause of Meniere's is:

3 main symptoms:

1. Vertigo
2. _____
3. Hearing _____

The patient may also complain of:

Best position during a Meniere's attack:

Diagnosis:

Diet:

Treatment:

1. Medical
2. Surgical

NCLEX Prep:

Avoid:

DIABETES OVERVIEW

A.) _____ is a _____
 _____ in which the _____ levels
 are too _____.

Two types of Diabetes Mellitus	Diabetes Type 1	Diabetes Type 2
1)		
2)		
3)		
4)		
5)		1. 2. 3. 4. Oral Non-Insulin Medications

B.) Signs/Symptoms:

DIABETES OVERVIEW

B.)

- 1.
- 2.
- 3.

C.) Complications of Diabetes.

Normal blood glucose level is: _____

Hyperglycemia	Diabetes Type 1	Diabetes Type 2
Cause	Diabetic Ketoacidosis (DKA)	Hyperosmolar, Hyperglycemic Non-Ketotic (HHNK)
Signs	A- Acidosis ph less than 7.30	
Treatment	Which IV fluid is best, dextrose water or 0.9% Sodium chloride?	

DIABETES OVERVIEW

D.) Complications continued

Hypoglycemia	Diabetes Type 1	Diabetes Type 2
Cause	1. 2. 3.Excessive exercise	Insulin overdose Too little food Excessive exercise
Signs	1. Feeling irritable	1. Shock
Treatment		

*The practical nurse treatment for the unconscious client is glucagon via IM injection. The solution is clear. The normal dose is 1 mg for adults and 0.5 mg for children.

Laboratory Value

1.) Hemoglobin A1C is a blood test used to determine blood sugar control over _____ months.

You want it to be less than _____.

E.) Insulin Types and Actions

Types	Generic Name	Onset	Peak	Duration
Rapid	<i>Insulin Aspart</i>		1 hour	3 hours
Short acting		Give this 30-mins. before meal		
Intermediate Acting solutions				
Long Acting	Glargine Determir			When do you give glargine? Can you mix with other insulins? Do you shake this insulin?
70/30 mixtures 70-Regular 30-NPH	Human 70/30 Novolog 70/30	30 minutes	2 hours	12 hours

DIABETES OVERVIEW

Anti-hyperglycemia Oral Agents

Medications	Side Effects	Client Education
1. _____		

TEACHING NOTES

1.
2.
Vomiting after taking PO medications?
3.

4.

5.

6.

DIABETES OVERVIEW

Clinical Judgement Questions

1. A client with type 2 diabetes mellitus is scheduled for hip surgery in 12 hours. The client is currently NPO. The healthcare provider writes an order to continue home medications. The client takes metformin PO in the mornings. Which is the **most** appropriate intervention by the registered nurse?

1. Call the healthcare provider to request a snack to be given in the morning.
2. Give the morning medication after pharmacy sends it up to the unit.
3. Call the healthcare provider and clarify the metformin order.
4. Give IV insulin to regulate the blood glucose levels.

2. A client has received discharge teaching about insulin and a diabetes mellitus type II diagnosis. Which of the following is the **most** important teaching point for the registered nurse to begin with?

1. Asking the patient to demonstrate how to measure and administer insulin.
2. Explaining ways of storing insulin and utilizing syringes.
3. Giving information on how many calories are needed with each meal.
4. Teaching about the long term side effects of poor insulin control.

3. A six-years-old boy is taken to the emergency department with lethargy. The boy has diabetes mellitus. He appears to be dehydrated—his eyes are sunken and mucous membranes are dry. He has a two week history of polydipsia, polyuria, and weight loss. Which blood gas value would you expect?

1. respiratory acidosis
2. respiratory alkalosis
3. metabolic acidosis
4. metabolic alkalosis

DIABETES OVERVIEW

Advanced Clinical Topics: External Insulin Pumps

1. _____ insulin pumps are _____ operated devices for type _____ and _____ diabetes mellitus.
2. Insulin pumps use _____ or _____ action insulins.
3. Insulin pumps have soft _____ inserted under the skin.

Advantages:

No need for _____.

Client can program the insulin pump to deliver a specific amount of _____.

Insulin pump can give _____ and bolus doses.

Pumps keep _____ of the insulin administered.

Disadvantages:

Skin irritations are _____.

Wearing the pump is a constant reminder of the _____.

Pump can be expensive.

Question:

Does the client still need to check their blood glucose levels?

Does the client need to rotate the pump insertion site?

Can a client with an insulin pump have an MRI?

ENDOCRINE REVIEW

Hyper _____	Hypo _____
<i>Signs:</i>	<i>Signs:</i>
<i>Treatment:</i> Diet: Option 1: Radioactive iodine therapy <i>Precautions:</i> Or Option 2: Thyroidectomy	<i>Treatment:</i> Diet: Hormone replacement
<i>Thyroid Emergency:</i> Treatment:	<i>Watch For:</i> Nursing Care:

ENDOCRINE REVIEW

Adrenal Disorders

The adrenal glands are located:

The adrenal glands help us:

1) Addison's disease is too little by the adrenal cortex

Signs of Addison's:

Lab profile

Client with Addison have a decreased:

Treatment:

2) Cushing's Syndrome "too much" by adrenal cortex.

Signs/symptoms:

Lab profile

Treatment:

(Remember never abruptly stop taking steroids teach client to taper the drug off)

THERAPEUTIC COMMUNICATION

The purpose of using these strategies is to help your client express their feelings more effectively.

Don't Do This:

Use words like bad, good, wrong or right

On NCLEX Choose.....

Look for:

Parameters

Age	Hold
	Less than
	Less than
	Less than
	Less than

THERAPEUTIC COMMUNICATION

IMPORTANT

Medication	Antidote

Needle Information

Route	Skin layers	Gauge	Length
	Epidermis, dermis and into the subcutaneous fat		

PSYCHOLOGICAL CONCEPTS

1. _____ is an acute _____ change that is reversible with _____.

Causes:

S	
I	
D	
E	

Assessment note:

Memory Impairment Note:

Treatment:

NCLEX Point: Research shows that even experienced nurses confuse the symptoms of delirium with dementia.

2.

1.

2.

3.

The most common form of dementia is _____.

Critically Think

Is Alzheimer's disease considered a mental illness?

Know the _____ of Alzheimer's Disease and also find these in Quick Facts

1. _____ - inability to use an object correctly, client is unable to recognize what it is for.

2.

3. _____ - inability to communicate

4. _____

3. Stages of _____

1.

2.	A's Agnosia Alexia Aphasia Apraxia
3.	Incontinence of bowel and bladder Further decline in cognitive and psychomotor coordination.

Treatment

_____ and _____ should be provided by the nurse.

When _____ is not appropriate then

_____ is the next action.

NCLEX Tips: Other Nursing Interventions

1. Maintain a strict schedule to promote a sense of security
2. Keep a calendar/clock in sight.
3. Keep familiar objects such as family photos and objects from the past in the client's environment.
4. Address the client's caregivers to provide support.

4. _____

These clients are _____ to _____.

PSYCHOLOGICAL CONCEPTS

	= Extreme Sadness	= Extreme elation
Signs		
Similarities		
Treatments The treatment is the same for both.	Laboratory test:	

Critically think: Circle the diagnosis based on the symptom.

1. Difficulty accepting compliments	Depression	Mania
2. Bizarre dress	Depression	Mania
3. Euphoria	Depression	Mania
4. Fear	Depression	Mania
5. Impulsiveness	Depression	Mania

NCLEX Safety Point	Depression: Nursing Interventions:
	Mania: Defensive and against authority

PSYCHOLOGICAL CONCEPTS

5. _____ - can't tell the difference

between _____ and _____.

Disease is _____ and requires _____ treatment.

Positive Psychotic Symptoms

Negative Signs: mute catatonic, suicidal, homicidal

Treatment:

Medical: Antipsychotic medications

Environmental: Nurse behavior

Use _____

Sit in silence

Set _____

Nursing Care:

1. Always keep in mind _____.
2. _____ their _____.
3. Present _____.
4. Set _____.
5. Avoid _____ the _____.

PSYCH MEDICATIONS

1.) _____ - are used to reduce anxiety

Medication Examples

- 1.
- 2.
- 3.
- 4.

1. _____ term use only.
2. _____
3. Safer to use in elderly than typical anti-psychotic medication
4. Monitor clients with: _____.
5. Monitor ataxia, liver function, and bone marrow suppression.

Benzodiazepines can also be used as:

- 1.
- 2.
- 3.
4. Treatment for alcohol withdrawal symptoms

Side Effects:

A-

B-

-If clients overdose on benzodiazepines give?

Client Education Points Self-Reading Activity	1. Give at bedtime. 2. Teach clients do not mix with alcohol. 3. Gradually taper off medication do not stop abruptly.
--	---

2. _____.

_____ is used to treat insomnia.

Points to know:

Treatment is short term and should last _____ days.

Side Effects:

PSYCH MEDICATIONS

Client Education Points	1. Give immediately before bed 2. Can be given with food but for best effect take on empty stomach.
--------------------------------	--

Antipsychotic Medication

Typical Antipsychotics

Two Types:

1.) Phenothiazines-

Examples:

Route:

Side Effects: ABCDS + _____

Note: Tardive dyskinesia is considered an extrapyramidal effect.

Nursing Assessments:

1. Monitor for _____.
2. Monitor _____ especially temperature.
3. Teach clients to lay flat for _____ after administration.
4. Monitor for _____.
5. Phenothiazines takes _____ to begin working.

2.) Non-phenothiazine

Monitor client for:

_____ + _____

PSYCH MEDICATIONS

What drug can we give to lessen side effects? Benztropine

We have to tell patients that: Side effects are normal and expected. They have to keep taking drugs or the psychotic symptoms will return.

NCLEX Psych Emergency

_____ - an adverse reaction to anti-psychotic drugs.

Signs: _____ + _____ + _____

Nursing Care:

_____ - also used to treat psychotic symptoms but newer medications

Examples:

Atypical antipsychotics have less extrapyramidal symptoms

Monitor your patient for _____ .

Atypical antipsychotics can cause:

- 1)
- 2)
- 3)

NCLEX Pro-Tip: Typical Psych medications can also be called:

Atypical Psych medications can also be called:

NCLEX Psych Emergency

_____ -

Signs:

Nursing Care:

2.) Antidepressants

1. _____

These drugs block M.A.O. enzyme that breaks down epinephrine, dopamine, and serotonin which leads to depression. They also block tyramine which puts clients at risk for a hypertensive crisis.

MAOI's are not used as often due to various drug and food interactions

PSYCH MEDICATIONS

Examples:

- 1.
- 2.
- 3.
- 4.

Side Effects: ABCDS is seen in vital sign changes along with INSOMNIA

Diet Restrictions: See Tyramine diet chart

Client Teaching:

Medication may take 4-6 weeks to work.
Never give MAOI's with Tricyclics or SSRIs

Tyramine Restricted Diet

Meats	
Grains	
Fruits	
Vegetables	
Dairy	
Sweets/Oils	
Condiments	

PSYCH MEDICATIONS

Antidepressants

2. _____

These drugs inhibit the reuptake of serotonin in the treatment of depression

Medication	Teaching
Fluoxetine	

Contraindications:

Client Teaching:

1) St. John's Wort-

2) All S.S.R.I. may cause SAD head if too much is taken.

Symptoms:

S

A

D

Head

3. _____ also used to treat depression also increases serotonin, and norepinephrine

Examples:

- 1.
- 2.
- 3.
4. Imipramine
5. Clomipramine

PSYCH MEDICATIONS

TCAs can be used for childhood: enuresis

Side Effects of TCAs:

Cardiac changes: Tachycardia, orthostatic hypotension EKG changes
(Example long QT intervals)

Client education:

- 1.
2. Do not consume _____
3. May take _____ to achieve therapeutic levels.
4. Take medication appropriately:

ECG OVERVIEW



Depolarization or Repolarization? _____

What does it mean? _____

Why do we need to know it? _____

1. Start with the word _____
2. When something is "polarized" it means:

There are special cells in the heart that generate electrical activity.

Electrical Activity Starts in the Sinus Node located in Right Atrium.

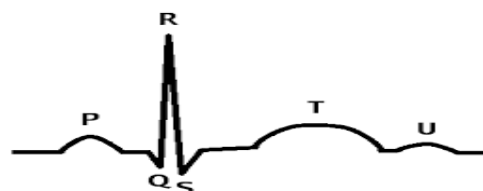
Depolarization means-

Ions move through channels. The 3 channels are:

Movement of ions across cell membrane causes:

Repolarization means-

During repolarization cardiac muscles are relaxed.

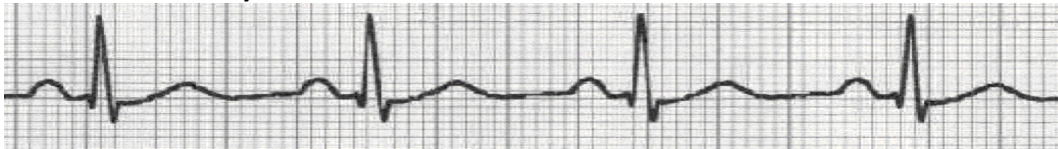


ECG OVERVIEW

Parts of the ECG	What is happening?
P wave	
	atrial repolarization and relaxation
	Ventricular repolarization and relaxation

Note: EKG/ECG only shows you electrical activity not muscular activity.

1. Normal Sinus Rhythm



Can you circle each p wave on this ECG strip?

Can you circle each QRS complex on this ECG strip?

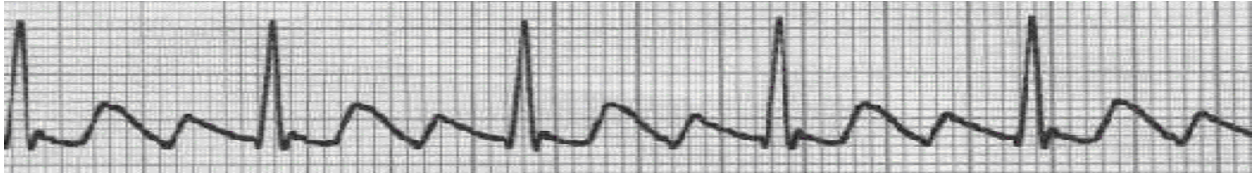
Normal sinus rhythm =

1. What is the rate?	
2. What is the rhythm?	
3. Is there a P wave before each QRS?	
4. Are the P waves upright and similar?	
5. What is the length of the PR interval?	
6. What is the length of the QRS complex?	

Rules: _____

ECG OVERVIEW

2. _____



1. What is the rate?	Atrial: 250-400 bpm Ventricular: variable
2. What is the rhythm?	Atrial: regular Ventricular: irregular
3. Is there a P wave before each QRS?	Normal P waves are absent
4. Are the P waves upright and similar?	
5. What is the length of the PR interval?	
6. What is the length of the QRS complexes	0.06-0.12

These NCLEX patients have Atrial Flutter:

Valve disorder (mitral)

Thickening of the heart muscle

Ischemia

Cardiomyopathy

COPD

Emphysema

NURSING INTERVENTIONS FOR ATRIAL FLUTTER

_____ – treatment of choice for NCLEX!!!

**Slow the ventricular rate by using: diltiazem, verapamil, digitalis, or beta blocker.

Heparin to reduce incidence of thrombus formation.

3. _____

ECG OVERVIEW



Ventricular Tachycardia =

1. What is the rate?	Atrial: Q Ventricular: 100-200
2. What is the rhythm?	Regular
3. Is there a P wave before each QRS?	Absent
4. Are the P waves upright and similar?	Absent
5. What is the length of the PR interval?	Not measurable
6. What is the length of the QRS complexes	Wide greater than 0.12 sec

Assess first if the client is:

Stable =

Unstable =

These patients have ventricular tachycardia:

Cocaine users

Chest trauma

DIGOXIN takers

Enlarged heart hx

Hypokalemia or low serum potassium

Treatment:

Never pick _____ as a treatment for

ventricular tachycardia because it:

What's the difference?

C	
D	

EKG OVERVIEW

4. _____

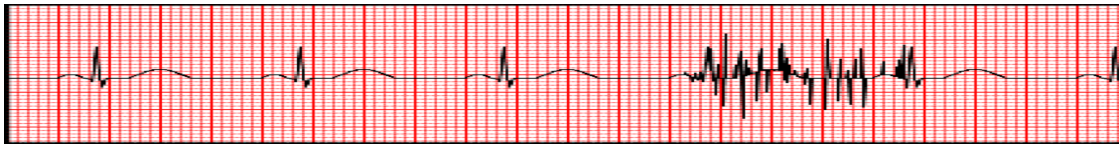


Asystole =

Treatment:

*If the monitor says asystole but the patient is clearly alive then check for electrode placement

5. _____



Artifact-

Do not put electrodes over hairy areas or bony prominences

Possible causes are:

Treatment:

NCLEX Advanced Topics: Heart Blocks



First Degree Heart Block

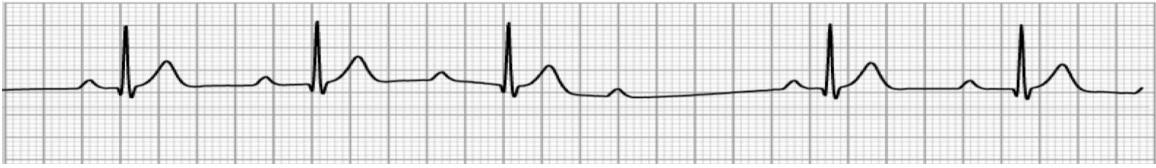
The PR interval is _____.

Is this an emergency?

What is the treatment?

What medications can cause first degree heart block?

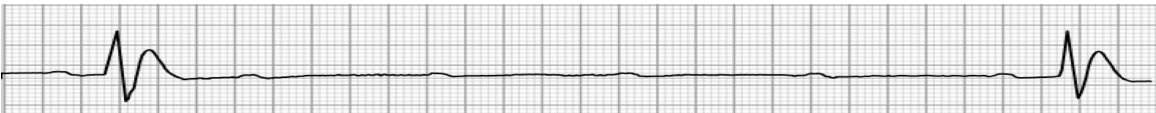
EKG OVERVIEW



Second degree heart block type 1 (Mobitz Type 1 or Wenckebach)



Second degree heart block type 2 (Mobitz Type 2)



Third degree heart block

Critical Thinking Question.

A registered nurse is caring for a client on continuous electronic heart monitoring. The client's electrical heart rhythm displays a progressively longer PR duration with a non-conducted p wave. Which type of heart block should the nurse document?

1. First degree heart block
2. Second degree, type 1
3. Second degree, type 2
4. Third degree

ISOLATION PRECAUTIONS

A.) Universal Precautions

1.
2.
3.

B.)

--

Contact Disease:

C.) Droplet Precautions

Droplet Disease:

Can the client's door stay open with Contact Precautions?

Can a client's door stay open with Droplet Precautions?

D.)

Airborne Disease:

Varicella can be replaced with disseminated herpes zoster or shingles.

ISOLATION PRECAUTIONS

Can the clients door stay open with airborne precautions?

Cohort Tip:

Disease	Precaution
AIDS	
Vaginal Yeast Infection	
Diarrhea	
Mononucleosis or Epstein Barr Virus	
West Nile Virus-transmitted by mosquitoes	
Hepatitis C	
MRSA- staph infection	
C-difficile	
Rota virus	
Shigellosis	
Head Lice	
Epiglottitis	
Influenza (seasonal)	
Rubella (German Measles)	
Whooping Cough (Pertussis)	
Meningitis	
Varicella (Chicken Pox)	
Monkey Pox	
Rubeola (Measles)	

Remember cohort client with same precaution

ISOLATION PRACTICE QUESTIONS

1. A nurse manager reports for duty and has to evaluate each nurse's assignment. A nurse has a client with A.I.D.S. and the nurse manager is evaluating his care by the healthcare team. She should intervene when she sees which situation?

1. A housekeeper cleans up spilled blood with a bleach solution.
2. A nurse takes the client's blood pressure wearing a mask and gloves.
3. A phlebotomist wears gloves to perform a blood draw.
4. A nurse attendant allows visitors to enter his room without masks.

2. A 7-year-old client is being admitted to the pediatric unit. What personal protective equipment must be placed outside the room when the nurse sees the client is diagnosed with varicella?

1. Gloves, disposable thermometer, private room, 6 fresh air exchanges per hour
2. Goggles, respiratory mask, gown, private room, gloves
3. Gown, private room, goggles, 6 fresh air exchanges per/hour, respiratory mask
4. Private room, goggles, respiratory mask, gloves, face/eye shield

3. What isolation precaution would you use for cutaneous anthrax?

1. Standard
2. Droplet
3. Contact
4. Airborne

4. What isolation precaution would you use for rotovirus?

1. Standard
2. Droplet
3. Contact
4. Airborne

ACCIDENT & ERROR PREVENTION

1.

A.

Why are children more at risk for medication errors?

Why do _____ work differently in _____?

1. Time of day

2.

3.

B. Falls

C.

D.

ACCIDENT & ERROR PREVENTION

Let's Talk Seizures: Seizures start in the brain

1. Place _____ on the _____ side.
2. Do not _____.
3. _____ stimuli is _____.
4. Do not put _____ in the client's _____.
5. Keep _____ and _____ at the _____.

2. Error Prevention

1. Restraints- are interventions used to reduce client harm.

2 Types: Chemical /Physical

Can an RN put on a restraint without an order?

If there is no _____ or _____ restraints
are considered _____.
_____ orders are called prescriptions.

Prescriptions have

- A.)
- B.)
- C.)
- D.)

2. _____ - Use _____ or _____ ink.

Writing is a legal document. It has to be _____.

_____ in charting-

3. _____ -form that details an unexpected event.

Reasons to write:

MANAGEMENT OF CARE

***Before watching the lecture. Study the definition box below.**

Registered nurses have several responsibilities when a client is in need of care.
The registered nurse can functions such as a:

1. Staff Nurse-Responsible for several clients.
2. Charge Nurse-Responsible for a team of nurses and their quality of care.
3. Nurse Manager-Responsible for a team of nurses, their educational growth, policy and procedures and to monitor the daily budgets of a location.
4. Case Manager-assesses, plans, implements, and evaluates a client's health and social service needs.

ReMar Pro-Tip: Remember registered nurses have the responsibility of discharging a client and that conversation begins during the admission process.

***Case management is not managed care.**

*Managed care are _____ used to lower or reduce the cost of _____.

Interdisciplinary Team

The goal is to:

The teams come together at a multidisciplinary conference.

Which clients need multidisciplinary team?

The client with complex _____.

The role of the registered nurse on the interdisciplinary team is to:

LEGAL ISSUES IN NURSING

A. Advanced Directives-_____ that allow clients to make decisions in _____.

They help to direct the client's _____ of _____.

	2.
DPA Notes Person has to be 18 years or older.	Document that allows to make decisions for 1. 2. 3.

***Know client's _____ status!!!

B) Informed Consent- _____ sure _____ is able to understand what is going to happen during a procedure, exam, test.

Need written consent to do	Can do without written consent
1. 2. Administer Blood 3. Anesthesia	1. 2.

LEGAL ISSUES IN NURSING

Who responsibility is it?

Clinical Judgement Scenarios:

1. What if a client's attorney requests information?
2. What if the nurse is caring for a child and the parents are away can consent be obtained over the phone?
3. Can a client have a procedure done even though they refuse to sign a written consent?

What is the difference?

When nurses receive assignments they have to take a report because they become responsible for that client's total care.

RN

Provide reassurance?

DELEGATION PRACTICE QUESTIONS

1. Who would be the most appropriate to feed a confused elderly client who can become combative?

- 1.) RN
- 2.) LPN/LVN
- 3.) Nurse's Aide
- 4.) Physical Therapist

2. Who would be most appropriate to assist a client ambulating down the hall for the first time after a hip replacement?

- 1.) RN
- 2.) LPN/LVN
- 3.) Nurse's Aide
- 4.) Speech Therapist

3. As part of the nurse's aide daily tasks she should complete the following tasks?

Select all that apply.

- 1.) Bath the client _____
- 2.) Provide assistance with ambulation _____
- 3.) Chart and calculate intake for shift _____
- 4.) Obtain sputum specimen _____
- 5.) Assist with feeding _____

4. A nurse is caring for a client with increased intracranial pressure, the client suddenly becomes unresponsive. A code is called and the nurse calls for assistance, who is most appropriate to gather the code cart?

- 1. Another registered nurse
- 2. A licensed practical nurse
- 3. A nurse's aide
- 4. The charge nurse of the unit

PRIORITIZATION

All the answers will seem right but only one is the PRIORITY!

1.) A new nurse working in the emergency room department reports for duty. Four patients arrive at the same time, who should she see first?

- A. A 100-years-old female with a temperature of 102, chills, and diarrhea.
- B. A 55-years-old male with severe abdominal pain from a kidney stone.
- C. A 2-years-old female with a heat rash complaining of itching.
- D. An 89-years old female with a poor gag reflex.
- E. A client with a gunshot wound to the right upper extremity.

Don't let NCLEX distract you with: _____ or _____

Only think about what is happening:

Look for the patient who is going to:

REVERSE PRIORITY

- 1. Least amount of _____
- 2. Least _____
- 3. Condition is plainly stated.

PRIORITY PRACTICE QUESTIONS

1.) A nurse is working in the emergency department at your local hospital. Four patients approach the triage desk at the same time. List the order in which you will assess these clients.

- A. 50 years old female with moderate abdominal pain and occasional vomiting.
- B. A 9 years old irritable female with nuchal rigidity, petechiae, and a fever.
- C. A 22 years old jogger with a twisted ankle, having a pedal pulse and no bruising.
- D. A 19 years old male with a bandaged head wound.

2.) A nurse is working in the emergency department. She has 4 clients which should she see first?

- A. A 79-year-old female with her right arm in a sling. She states she was walking out to get her mail when she tripped on an uneven sidewalk and fell onto her right side. She denies hitting her head or losing consciousness. She has a positive radial pulse in the right arm, there is some deformity noted.
- B. A 53-year-old male holding his left hand with his right hand and states "I was working on my horse trailer, trying to connect the trailer to the truck when I dropped the hitch and it landed on my hand." Patient has a crushing injury to his 2nd and 3rd fingers on the left hand.
- C. A 67-year-old male with slurred speech and carrying a mason glass jar with a snake in it. He states he was drinking with his buddies when he saw a snake in the driveway. He was trying to catch it when it bit him on the right hand. The snake is a water moccasin.
- D. A 79 year old male brought to the ER from the nursing home after a fall. He stood up in his wheelchair without assistance and attempted to walk. He stumbled and fell onto his left side and did not hit his head or lose consciousness. There are no injuries present.

3.) These clients present to the ED complaining of acute abdominal pain. Prioritize them in order of severity.

- A. 22 years old college student complaining of severe, intermittent cramps with three episodes of watery diarrhea, 2 hours after eating.
- B. A 22 years old female with a low-grade fever, left lower quadrant tenderness, nausea and an inability to eat for the last 3 days.
- C. A 12 years old female with moderate left upper quadrant pain, vomiting small amounts of yellow bile, and worsening symptoms over the past 3 days.
- D. A 56 years old male with a pulsating abdominal mass and sudden onset of pressure-like pain in the abdomen and flank within the past hour.

TIPS TO MASTER NCLEX

1. Content over _____
2. Forget the real _____
3.
4. Give yourself permission not to know _____
5. Confidence is _____.
6. Do not study the day before the exam.
7. You didn't come this far to Fail!

TEST DAY TIPS

- A. Know the location (parking, room, etc.)**
- B. Pack everything you need the night before.**
- C. Do not study the night before.**
- D. Eat breakfast and dress in layers.**
- E. Do not go in expecting to stop at minimum questions!**

CONGRATULATIONS – YOU’VE COMPLETED THE NCLEX VIRTUAL TRAINER!



You're one step closer to sitting for NCLEX! As I said in the introduction to this book, I'm confident that if you have studied the content and honored the process that you can pass NCLEX!

Even if you are feeling a bit anxious I promise you it is normal! This test is important and I know you feel pressure. But you CAN pass NCLEX– in fact, I want to see your testimonial video on the other side of NCLEX where you're smiling or sharing tears of joy as soon as you see your name and those big letters (PN or RN)! You deserve that moment and know that your success will not only bless you but it's going to bless everyone around you as well.

SO WHAT'S NEXT!

At this point you should have:

- Completed all of your NCLEX Virtual Trainer Videos
- Taken all quizzes with a 95% or higher competency rate
- Completely filled out your Student Workbook
- Memorized the Quick Facts for NCLEX book
- Re-reviewed all of your notes to access for weak areas

If you can say yes to these things your foundation is strong and you should head over to the ReMar Question Bank and continue to test your knowledge.

Your last two weeks of studying should be daily doing practice questions to prepare your mind for the actual exam. Attempt to do every question in your ReMar Nurse Question Bank and your computer adaptive testing as well. Your scores will reflect your knowledge. You may want to purchase additional practice tests to help increase your level of preparedness before testing.

Your nursing license is waiting on you but you have to be prepared in order to receive it!

So again if you can say yes to the following points above head to the question bank if you still don't feel sure **you may need to extend your VT access** in order to go back and review your weak areas before sitting for NCLEX! Which is a good thing especially as a repeat tester or foreign nurse. If you need help with access to the Virtual Trainer and need help email Team ReMar at Support@ReMarReview.com

Again we are looking forward to seeing your testimonial video, your time to become a nurse is now! You Can. You Will. You Must Pass NCLEX!